Example 21: Regulation of food and drugs

In the field of food safety, regulation in India is weak. A market failure, asymmetric information, is present when buying boxes for food storage and transportation. The consumer cannot know the extent to which the box is made of harmful substances.

Lacking such regulatory capacity, the typical plastic box for food storage and transportation, that is sold in India, may contain harmful substances. One easy solution *for a consumer* is to buy such plastic boxes from places where food safety regulation is strong. As an example, perhaps Tupperware plastic boxes sold in India benefit from US FDA regulation on food safety. When this is done, the Indian consumer is free-riding on the US regulatory system.

In the field of medicines also, the same market failure (asymmetric information) is present. When we buy medicines in India, there is a risk that these are ineffective or counterfeit, owing to weak regulation on drug safety.

As with Tupperware plastic food storage boxes, one could think of Indian consumers buying medicines which benefit from the oversight of mature democracies and thus free-ride on their regulatory capacity. In general, a bottle of aspirin purchased in a developed country is superior to a bottle of aspirin purchased in India.

These pathways are hampered by the Drug Price Control Order (DPCO) which forces very low prices for drugs in India.

In the field of drugs and food, the US and EU authorities run active inspection programmes through which certain factories in India obtain the rights to produce drugs or food products which can be exported to the EU or the US. There is a natural opportunity for public policy in India to free-ride on this work.

Perhaps an Indian drug safety regulator can say that the privilege of selling drugs in India is limited to factories that have earned the permission to export drugs to mature democracies. Perhaps there can be a way for